Royal Devon University Healthcare NHS Foundation Trust

Standard Operating Procedure for
Preconception Care for Women with Existing Medical Conditions
Eastern Maternity Services

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Note: This document has been assessed for any equality, diversity or human rights implications			

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1. INTRODUCTION

This document sets out how to optimise preconception care for women who have pre-existing medical conditions.

2. PURPOSE

The SOP has been written to standardise the care offered to women with pre-existing medical conditions.

3. REFERRALS

For women with **pre-existing medical conditions** please refer to the obstetric team by contacting the lead consultant for maternal medicine, preferably by a letter on Epic. If unsure please refer or phone for advice.

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If the referral is for a woman with diabetes with no other complications then this should be directed to the Diabetes Specialist Nurses who run preconception clinics on Tuesday afternoons in the Diabetes Centre.

If the referral is about a previous obstetric issue or birth complication then please first consider referral for a Birth Conversation with a midwifery matron or refer to a general obstetric consultant.

Psychiatric conditions and medications can be discussed with the perinatal mental health team consultants and pharmacist, who can also offer pre-conception counselling.

4. TRIAGE

Referrals will be triaged by the lead consultant for maternal medicine.
This may result in:

- A letter – to answer a query or give advice about medications
- Referring to another department/team e.g. genetics or perinatal mental health
- Telephone consultation
- Joining a clinic with a specialty team e.g. in renal clinic
- Face to face appointment in a designated preconception clinic in the Centre for Women's Health (some cases may need more than one appointment)

Women who are invited for a face-to-face appointment at the Centre for Women's health (CWH) will be sent a letter – See APPENDIX A

5. PRECONCEPTION CLINICS IN CWH

Preconception clinics occur approximately every 2-3 months. Each clinic has five 40-minute appointments.

Registrars with an interest in maternal medicine are welcome to join these as a learning opportunity. Please arrange this in advance and do some background reading around the relevant cases before attending.

Patient should have their BMI calculated, urine dip and BP on arrival.

Assessment and documentation will be in line with APPENDIX B.

Please see southwestmaternalmedicine.nhs.uk for patient information videos about the following:

- Asthma
- Epilepsy
- Hypothyroidism
- Inflammatory Bowel Disease
- Autoimmune arthritis
- Chronic kidney disease
- Cystic Fibrosis
- Obesity

APPENDIX A:

Letter to woman with a pre-existing medical condition – specimen wording

You are being offered an appointment to discuss your condition and medications before trying to conceive.
This appointment will be held in the gynaecology out-patient department.

You will have some basic observations taken such as your weight and blood pressure but it is unlikely you will need to have a physical examination during the appointment.

You are welcome to bring someone with you to the appointment but please do not bring children.

It would be helpful to see a list of your current medication and if there are letters and test results from another hospital then please bring these too.

If you are unable to attend please phone to cancel or rearrange this appointment.

It is advisable to use reliable contraception until your medical condition is optimised and you have discussed this in the clinic. If you do become pregnant whilst on the waiting list then please let us know and we can arrange for you to be seen in the hospital antenatal clinic rather than the preconception clinic.

APPENDIX B: Outline for the preconception clinic consultation

Age

Parity

Condition/s

BMI

Relationship/partner details

Disease control
A well woman with good management of underlying condition will have better pregnancy outcomes for her and her baby.
Consider – what is best treatment when not pregnant and what would be the reasons for not doing this?

Top to toe thoughts about which organ systems are affected

Are there any antibodies which could affect pregnancy?

Pregnancy implications on disease ?flares

Disease on pregnancy e.g. increase risk of IUGR, preterm birth, PET

Medication review

Genetics if relevant

VTE assessment

Consider anaesthetic referral if relevant

Proposed plan for care in pregnancy – consider each trimester and postpartum period

Reliable contraception until everything optimised to conceive

Preconception folic acid and correct dose

Baseline bloods e.g. Hb, HbA1c, renal if required

Lifestyle advice re BMI, smoking cessation if relevant

Please consider using smartphrase in Epic:
"JBPRECONCEPTIONCOUNSELLINGTEMPLATE"

A letter will be written to the referring clinician and GP with a copy to the patient once all the information and advice has been collated. This may be after discussion at an MDT.